

Cervical Cancer Screening Preferences Among Women in the U.S.

Each year in the United States (U.S.), there are over 13,000 new cervical cancer diagnoses and about 4,000 cervical cancer deaths. Cervical cancer is preventable and highly curable if caught early. However, cervical cancer screening rates have been declining in recent years, with screening coverage among women aged 21–65 decreasing from 86.5% in 2000 to 75.8% in 2023. This reduction in screening adherence is concerning because most cervical cancer cases and deaths in the U.S. occur among individuals who are not up to date with screening guidelines. Additionally, research has shown substantial differences in cervical cancer screening by characteristics such as age, race/ethnicity, socioeconomic status, and rurality, which contribute to disparities in cervical cancer incidence and mortality. Efforts are therefore needed to increase cervical cancer screening rates, particularly among underserved women.

Offering women the option to collect the sample of cells needed for cervical cancer screening by themselves at home (rather than by a health professional at a clinic) could help increase screening uptake. At-home self-sampling would address many of the known barriers to clinic-based testing, including limited access to care, logistical challenges (e.g., lack of transportation), and embarrassment or discomfort with pelvic examinations.

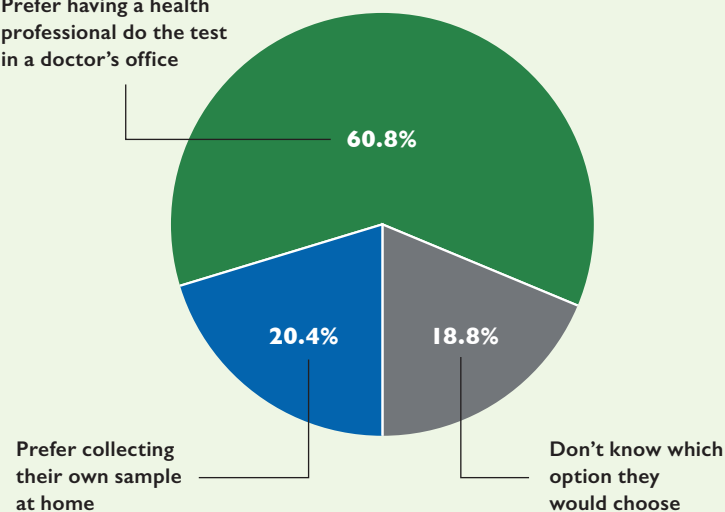
Studies have shown that at-home self-collection kits are a feasible, acceptable, and potentially effective approach for improving cervical cancer screening participation among underscreened women, with many women reporting positive experiences with self-collection. However, some women report concerns about self-collection, including perceptions of lower test accuracy, uncertainty about their ability to perform self-collection correctly, and worry about pain or physical discomfort. Acceptability is a key determinant of uptake, making it important to understand the perspectives of screening-eligible women about different sample collection approaches, especially because studies have shown that attitudes toward self-collection for cervical cancer screening may vary by factors such as screening status, race, ethnicity, education, income, and sexual orientation. Understanding women's perceptions of different sampling approaches is also critical given that national screening guidelines are increasingly endorsing self-collection as a screening option.

Quick Facts

- Most cervical cancer cases and deaths in the U.S. occur among individuals who are not adherent to screening guidelines.
- Cervical cancer screening rates among U.S. women decreased from 86.5% in 2000 to 75.8% in 2023.
- Home-based self-sampling could increase screening uptake by addressing barriers to standard screening conducted in a clinic. However, some women report concerns about self-collection, including perceptions of lower test accuracy and uncertainty about their ability to perform sample collection correctly.
- In 2024, 20.4% of screening-eligible women reported preferring home-based self-sampling for cervical cancer screening, 60.8% preferred sampling performed by a health professional at a clinic, and 18.8% were not sure which option they preferred.

Preferred Sample Collection Approach for Cervical Cancer Screening Among Screening-Eligible Individuals*

Prefer having a health professional do the test in a doctor's office



*Note: Women aged 21 to 65 were considered screening-eligible for the purpose of analysis. Source: Analysis of HINTS 7 (2024) data in Fokom Domgue et al. (2026).

Factors Associated with Cervical Cancer Screening Sampling Approach Preferences

A recently published analysis of HINTS 7 (2024) data found that among cervical cancer screening-eligible women aged 21–65, 20.4% would prefer to do the screening test by themselves at home (home-based self-sampling), 60.8% would prefer to have a health professional do the test in their office (clinician-collected sampling), and 18.8% were uncertain which option they would choose. Logistic regression modeling showed that compared to non-Hispanic White women, non-Hispanic Black women had lower odds of preferring home-based self-sampling over clinician-collected sampling, whereas individuals who reported experiencing prejudice or discrimination in medical settings had higher odds of preferring at-home self-sampling.

Additionally, Hispanic women were found to have lower odds of being uncertain about their preferred sampling approach (relative to preferring clinician-collected sampling) than White women. Women with household incomes below \$50,000 had higher odds of being uncertain about their preference compared to those with incomes of \$75,000 or more. Compared to women reporting no health care visits in the past 12 months, those with 2 visits or more had lower odds of being uncertain. Finally, women who reported less trust in the health care system had higher odds of being uncertain about their preference compared to those with greater trust.

Women who reported preferring at-home self-sampling or being unsure about their preference were also asked why they might consider at-home self-sampling. Frequently selected reasons included privacy (selected by 54.9%), not wanting to take time off work (35.1%), and wanting to avoid embarrassment (33.4%).¹

¹Note: Respondents could select more than one reason, as well as “other.”

How Can This Inform Your Work?

HINTS data suggest that although most women currently prefer clinician-collected sampling for cervical cancer screening, a substantial minority would be interested in at-home self-sampling. Additionally, many women express uncertainty regarding these options, and preferences vary by demographic factors and health care experiences. Home-based self-sampling has the potential to increase screening uptake and reduce disparities, particularly among women who face logistical or psychological barriers to clinic-based screening. However, to increase the utility of at-home self-sampling, efforts to increase awareness of—and trust in—this sampling approach, as well as to improve confidence in performing self-collection, will likely be needed. Patient-facing communications about self-sampling could highlight its accuracy, address concerns about the possibility of pain or discomfort, and support sample collection self-efficacy, for example, through the use of simplified language and clear step-by-step instructions, along with visual aids or instructional videos.

Communication from health professionals may also play an important role in uptake of self-collection, as their recommendations have a strong influence on cervical cancer screening behavior, and their endorsement could help address patient concerns (e.g., regarding test accuracy). However, research shows that some health professionals have insufficient knowledge about self-sampling, are not confident in their ability to discuss it with patients, and may have their own doubts about the adequacy of this approach. This suggests a need for education efforts focused on health professionals to increase their comfort with self-sampling for cervical cancer screening and enhance their self-efficacy in discussing this option with patients (e.g., educational materials, online continuing medical education courses, Grand Rounds to share best practices). These efforts could help ensure that health professionals are prepared to engage in shared decision-making conversations with patients about their cervical cancer screening options.

About HINTS

hints.cancer.gov

The National Cancer Institute (NCI) created the Health Information National Trends Survey® (HINTS) to monitor changes in the rapidly evolving field of health communication. The survey data can be used to understand how adults use communication channels to obtain health information for themselves and their loved ones. HINTS data can also help practitioners create more effective health communication strategies. The HINTS survey has been fielded 17 times to date.

HINTS Briefs provide a snapshot of noteworthy, data-driven research findings that may be of interest to the HINTS community. The Briefs summarize research findings from recently published peer-reviewed journal articles analyzing HINTS data.

For More Information on Cancer

- Call the NCI Cancer Information Service at 1-800-4-CANCER
- Visit <https://www.cancer.gov>
- Download NCI publications at <https://pubs.cancer.gov/ncipl/home.aspx>
- Visit [Facebook.com/cancer.gov](https://www.facebook.com/cancer.gov) and <https://www.youtube.com/ncigov>

References Used in This Brief

- Fokom Domgue J, Chandra M, Oladoyin O, Desai M, Yu R, Shete S. Women's preferences for home-based self-sampling or clinic-based testing for cervical cancer screening. *JAMA Netw Open*. 2026;9(2):e2558841. doi:10.1001/jamanetworkopen.2025.58841.
- Tiro JA, Metcalfe S, Muthukrishnan M, et al. Promoting cervical cancer screening via a mailed HPV self-collection kit: reactions from screeners and non-screeners. *Patient Educ Couns*. 2026;142:109374. doi:10.1016/j.pec.2025.109374.
- Qin J, Martinez G, Holt HK, et al. Preferences among U.S. women for cervical cancer screening with self-collected specimens for human papillomavirus testing. *Obstet Gynecol*. 2026;147(3):306-312. doi:10.1097/AOG.0000000000006147.
- Roth S, Lott B, Garo J, et al. Taking health into your own hands: evaluating patient and provider perspectives of human papillomavirus self-sampling for cervical cancer screenings and opportunities for education. *J Cancer Educ*. 2025;40(5):733-740. doi:10.1007/s13187-025-02568-4.
- Mkuu RS, Tohme S, Rivero-Mendoza D, et al. Improving patient education materials for HPV self-collection: insights from women at high risk of developing cervical cancer. *Cancer Manag Res*. 2025;17:3273-3284. doi:10.2147/CMAR.S552282.

